blue of california

June 16, 2025

Dr. Mehmet Oz, MD
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
7500 Security Boulevard
Baltimore, MD 21244-8013

Submitted electronically via https://www.regulations.gov

Re: Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Dear Administrator Oz,

Blue Shield of California (Blue Shield) appreciates the opportunity to respond to the Centers for Medicare & Medicaid Services' (CMS) Health Technology Ecosystem Request for Information (RFI). Blue Shield is a nonprofit health plan offering health benefits coverage to individuals and groups throughout the State of California. Our mission is to ensure that all Californians have access to high-quality care at an affordable price.

We applaud the Administration's commitment to improving access to health care data and digital health tools to help patients and families make informed healthcare decisions, reduce the burden on clinicians and other providers, and lessen costs for businesses and taxpayers. As CMS and the Assistant Secretary for Technology Policy/Office of the National Coordinator (ASTP/ONC) contemplate their digital health priorities, we encourage your consideration of policies that accelerate the seamless, secure, and real-time exchange of electronic health information across the health care system and enable all Americans to access a longitudinal, real-time digital health record.

Like CMS, Blue Shield is committed to bringing health care into the digital age and breaking down barriers to the secure and timely exchange of health information. We are active participants in California's Data Exchange Framework (DxF), California's statewide data sharing agreement, which charts new territory by requiring health plans, hospitals, and clinicians (among other providers) to exchange or provide access to electronic health information in real time. The DxF was enacted in 2021 as part of AB 133, state legislation that Blue Shield endorsed. In addition, Blue Shield has made significant investments in California's health information exchange infrastructure and shares claims data with providers via Manifest MedEx, the state's largest non-profit health information network. Blue Shield also participates in regional health information networks, like San Diego Health Connect and the Los Angeles Network for Enhanced Services, and connects with providers via Epic's Payer Platform.

Blue Shield has taken meaningful steps to improve our members' visibility into their health through the launch of our Member Health Record tool. Our goal is for the Member Health

Record to serve as the single source of truth for our members' medical history – bringing clinical data from multiple providers together with pharmacy and claims information into one longitudinal digital health record that is free to members. However, even with multiple avenues to participate in data exchange and a state mandate to reinforce data sharing, our ability to access member data in real time remains a challenge.

To address the challenges of unlocking the data created by our health care system, we recommend that CMS and ASTP/ONC:

- Require health care organizations to participate in real-time data sharing;
- Enforce statutory prohibitions on information blocking and implement additional compliance mechanisms to improve data sharing; and
- When data is shared, hold health plans accountable for establishing longitudinal, real-time digital health records for their members.

We have been advocating for several of these recommendations in Congress as part of a draft bill, *Comprehensive Digital Health Records for All Americans Act*, which is attached for your reference.

Our detailed comments follow.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

Despite the enactment of the *21st Century Cures Act (Pub. Law 114-255)*, our health care system still relies on antiquated faxes, photocopies, and CD-ROMs to share data. Patient health information remains trapped in disparate clinical and administrative systems and networks. While patients have a legal right to access their information under the HIPAA Privacy Rule, there is no guarantee that their information is integrated into a comprehensive, longitudinal digital health record. Even using patient-facing Application Programming Interfaces (APIs) – which offer an advanced technological approach to accessing data – patients are still required to recall the facilities and clinician practices that may have their medical information.

As noted in the RFI, "although the building blocks for a patient-centric digital health ecosystem are in place, the experience of most patients, caregivers, and providers is neither seamless nor simple. To get access to their data, patients have to track which providers they have seen and set up separate accounts and credentials with each portal." This is the shared reality of far too many Americans in our health care system.

<u>We recommend CMS prioritize adoption of digital tools or applications that enable patients access to the full picture of their health history as part of an integrated, comprehensive, and up-to-date digital health record. Specifically, we encourage CMS to effectuate regulations that (1) establish that patients have the right to access their own comprehensive, digital health record that is updated in real time or not later than 24 hours after new health information is available; and (2) make health plans accountable for compiling and providing their members electronic access</u>

to clinical, claims, and other electronic health information in an integrated, user-friendly digital health record.

Taking these steps would help address several inefficiencies and patient harms resulting from piecemeal and outdated information and, importantly, alleviate the burden that our current system puts on patients to hunt for their information from disparate record systems, portals, or apps. It would empower patients with access to their own information and improve their care experience.

As noted earlier, Blue Shield has taken meaningful steps to improve our members' visibility into the big picture of their health and alleviate the burden of navigating multiple physician and hospital portals by launching our Member Health Record. This platform is available on our Blue Shield member app and website and serves as a single source of truth for our members' medical history – bringing data from multiple providers all in one place. Members across Blue Shield insurance products can electronically access health records available to Blue Shield (including lab results, immunizations, and pharmacy information), historical health data shared with Blue Shield, and claims information. Importantly, the Member Health Record can be emailed, printed, and shared with an individual's care team and their loved ones. We believe this experience should be available to all Americans.

PC-2. c. What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

As noted above, one obstacle to patients accessing complete health information electronically is the lack of ownership in our health care system for curating and updating comprehensive, longitudinal digital health records in a manner that is "without special effort" on the part of patients. Current solutions do not meet this standard.

We recommend CMS require health plans to compile and make accessible comprehensive digital health records that integrate clinical, claims, and other available electronic health information, are updated in real time when new electronic health information is accessible to the health plan, and are available to plan members through a secure, web-based application, portal, or other platforms. Health plans have visibility into where their members access care and can assemble medical records from members' treating providers (as permitted under existing state and federal rules). In addition, when the Payer-to-Payer API is effective in 2027, health plans will be able to fill gaps in members' claims history by requesting – when authorized – historical claims data from their members' former health plans. Therefore, we believe health plans are well positioned to create, maintain, and make available comprehensive digital health records for their members.

Another obstacle to accessing complete health information electronically is the lack of will or cooperation within the health care industry – often by entities seeking a financial or competitive advantage – that inhibits timely and seamless data sharing. Additionally, mixed signals in what is currently required or allowed for sharing data among payers, clinicians, hospitals and other

health care entities results in inefficiencies and friction across the health care system, impedes the delivery of high-quality care, and inhibits patient access to their comprehensive digital health record.

With this in mind, we urge CMS to set clear expectations that data exchange is a shared responsibility across *all* stakeholders. Specifically, we recommend CMS issue regulations that require key health care organizations – hospitals, physician practices and medical groups, health plans, skilled nursing facilities, and clinical laboratories – to provide access to or exchange all electronic health information in real time or not more than 24 hours after the information is available for all required and permitted uses and disclosures under HIPAA.

Establishing consistent data sharing requirements across key health care organizations would make it easier for patients, providers, health plans, and other health care organizations to access and use electronic health information. It would also address the barriers health plans currently face in accessing members' clinical data for important healthcare operations use cases, including creating, maintaining, and making available comprehensive digital health records for their members. Ultimately, health data should be treated as a common good and participating in data exchange should be table stakes in our health care system.

TD-18. Information blocking: a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response.

As a health plan, we recently initiated – through our health information network, Manifest MedEx – a Healthcare Operations query to a Los Angeles-based hospital for three individuals who received care at the hospital and had claims adjudicated by Blue Shield. In all three cases, the hospital responded, "No patient found." The stated reason for the hospital rejecting the Healthcare Operations query was that their Epic system's feed to Carequality is not configured to respond to Healthcare Operations queries.

While we are working in good faith with this hospital provider, Epic, and Manifest MedEx to resolve this issue, it is an example of challenges we have faced accessing data via queries with a Healthcare Operations purpose code. We often run into resistance, misunderstanding, or technical barriers that impede our ability to access member data in a manner permitted under state and federal law. Moreover, because Healthcare Operations is subject to the minimum necessary standard under HIPAA, responding entities have discretion over the degree of data disclosed in response to Healthcare Operations queries. This subjectivity creates confusion, inconsistency, and friction in our health care system.

We encourage CMS and ASTP/ONC to work with health information networks, like the Trusted Exchange Framework and Common Agreement, to establish standard operating procedures and policies that (1) *require* responses to Healthcare Operations queries without permitting respondents to tack on additional response fees; and (2) detail a reasonable, standardized approach to the minimum necessary standard.

TD-18. Information blocking: b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

Blue Shield strongly supports the Administration's efforts to discourage information blocking and believes the mechanisms for enforcing the prohibition on information blocking must be meaningful and comparable across all stakeholders. Robust enforcement mechanisms are needed to reinforce ASTP/ONC's information blocking rules and ensure the free flow of electronic health information. We urge ASTP/ONC to work with the HHS Office of Inspector General to fully implement and enforce the statutory prohibition on information blocking, and publicly detail the actions being taken to address valid claims of information blocking.

We are encouraged that CMS and ASTP/ONC view their existing authorities as a starting point and not the end of the discussion on disincentives for engaging in information blocking. On that note, we recommend CMS consider alternative compliance mechanisms, such as requiring hospitals, larger medical groups (with 25 or more physicians), health plans, and developers of certified health information technology to publicly report on and attest to a standardized set of compliance metrics developed by CMS and ASTP/ONC. The survey could include questions on the implementation status of existing mandates (e.g., adoption of APIs), number of external queries received and responded to by HIPPA Purpose of Use, number of outbound queries sent by HIPPA Purpose of Use, standards they use, and metrics of data completeness and quality. Public reporting of this type of information would improve transparency into how organizations are engaging in data sharing and make it easier to identify which organizations are lagging or potentially engaging in information blocking practices.

PC-5. b. What changes would enable timely access to high quality CMS and provider generated data on patients?

We strongly encourage CMS to revisit and strengthen its Admission, Discharge, and Transmission (ADT) patient event notification condition of participation (CoP) for hospitals. ADT event notifications are a key source of information that support coordination of care and patient outreach by primary care and community providers as well as health plans. Hospitals have had over four years of experience complying with CMS' CoP and it is time to build on the existing requirement to improve the utility of ADT notifications.

We recommend CMS issue regulations proposing the following enhancements to ADT event notifications:

- Expand the minimum information in the ADT event notification CoP to include fields for the following data elements:
    - National Provider Identifiers for the (1) Sending Facility, (2) Attending Doctor (if available), (3) Referring Doctor (if available), and (4) Discharged Location;
    - Discharge Disposition;
    - Admit Reason; and
    - ICD-10 Codes for (1) Diagnosis; (2) Diagnosis Description; and (3) Diagnosis Type.

- We also recommend CMS expand ADT event notifications to apply to patients who present in emergency rooms and are subsequently discharged without being admitted, as well as patients who are admitted by hospitals in observational status.

These enhancements would greatly improve the usability of ADT event notifications, providing critical information to primary care providers and health plans to support timely follow up with the patient after a hospital visit and coordinate after care. We believe these changes would improve care transitions, reduce waste, improve patient outcomes, and would not be a significant additional burden on hospitals.

PC-5. a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)?

We encourage CMS to establish a centralized mechanism – in partnership with the Federal Trade Commission – to validate and designate "trusted" digital health products based on their adoption and use of industry-endorsed privacy and security standards, and for having consumer disclosure policies that provide transparency into how such products use personal health information for secondary purposes. We encourage any vetting process to evaluate applications based on their privacy and security practices, consumer education and marketing tools, the adequacy of their consent policies, the transparency of their terms and conditions, and the type of secondary uses they intend to carry out using personal health information. A "CMS-approved" marketplace for digital health products would provide a centralized, user-friendly place for consumers to select reliable, trusted digital tools for managing their health needs.

We also believe CMS has an opportunity to increase efficiency and uptake of the Patient Access API by establishing a centralized portal for API registration that could be used by CMS-vetted apps to automatically register with all health plan Patient Access APIs. This would reduce the need for point-to-point registrations between apps and health plans, reducing administrative burden and costs.

Without a federally supported vetting process, we are concerned about the potential for misuse and/or inappropriate disclosure of enrollees' personal health information by third-party apps and developers, which fall outside the reach of HIPAA. Lengthy terms and conditions written in legalese can leave consumers confused or unaware that they have authorized an application to use or sell their personal health information for secondary purposes. We believe that all entities, whether or not they are covered by HIPAA, must bear responsibility for protecting personal health information, clearly conveying to individuals how their health information will be used, and obtaining individuals' consent for secondary uses and disclosures of their health information.

Conclusion

We appreciate your continued work and attention to modernizing care delivery, improving access, and supporting innovation in our health care system. Thank you again for the opportunity to provide comments on the Request for Information. We would welcome the opportunity to respond to any follow-up questions you may have.

Sincerely,

Andy Chasin
Vice President, Federal Policy and Advocacy
Blue Shield of California